

MEDICAID HOME AND COMMUNITY-BASED WAIVER SCOPE OF
SERVICES FOR
**CHILDREN'S ATTENDANT CARE (SELF-
DIRECTED) ***

***Children's Attendant Care is only available to Medically Complex Children (MCC) waiver participants aged one to 21 with extraordinary care needs.**

A. Minimum Qualifications

1. Children's attendant caregivers must meet the following minimum qualifications:
 - a. Demonstrate an ability to read, write and communicate with the participant, responsible party and/or Employer of Record (EOR).

Exception: When the participant or responsible party is unable to identify a separate, qualified individual to serve as the EOR, the responsible party may be allowed to serve as the EOR and the children's attendant caregiver. Exception requests must be submitted by the MCC Waiver Nurse Case Manager to WES@scdhhs.gov.
 - b. Capable of aiding in activities of daily living; physically capable of performing duties which may require physical exertion such as lifting, transferring, etc. if necessary;
 - c. Capable of completing the tasks identified for the participant on the waiver care plan with participant and/or EOR;
 - d. Be at least 18 years of age;
 - e. Capable of following billing procedures and completing required paperwork;
 - f. Have no known conviction for any of the following:
 - i. abuse, neglect or exploitation of adults (as defined in the Omnibus Adult Protection Act, S.C. Code Ann. Title 43, Chapter 35) or of children (as defined in the Children's Code, S.C. Code Ann. Title 63, Chapter 7); any crime against another person;
 - ii. felony of any kind; Any crime concerning the misuse or abuse of public assistance program (including, but not limited to, fraudulently obtaining benefits, engaging in fraudulent billing practices, and embezzling or otherwise misusing public assistance funds in any manner). No exclusion for Medicare or Medicaid Programs;

- iii. Any crime concerning the misuse or abuse of any public assistance program (including, but not limited to, fraudulently obtaining benefits, engaging in fraudulent billing practices, and embezzling or otherwise misusing public assistance funds in any manner). No exclusion for Medicare or Medicaid Programs;
- iv. Upon request the attendant must provide references to the participant and/or responsible party;

g. Screening

Prior to the children's attendant caregivers' Medicaid provider enrollment date, the attendant must provide evidence of having a single Blood Assay for Mycobacterium tuberculosis (BAMT) or a baseline two-step Tuberculin Skin Test in the following manner:

- 1. With a documented negative Tuberculin Skin Test or a BAMT result within the previous twelve (12) months, the children's attendant caregiver may have a BAMT or a single Tuberculin Skin Test, administered and read, to serve as the baseline prior to provider enrollment.
- 2. Without a documented negative Tuberculin Skin Test or a BAMT result within the previous twelve (12) months, the children's attendant caregiver shall have a BAMT or a two-step Tuberculin Skin Test, administered and read, prior to Participant contact.

After known exposure to a person with potentially infectious tuberculosis disease without the use of adequate personal protection, the children's attendant caregiver must ensure his or her tuberculosis status is determined in a manner prescribed in the current Centers for Disease Control and Prevention and SC Department of Public Health tuberculosis guidelines. If TB is present, the children's attendant caregiver must report any known or suspected cases of TB disease to the local SC Department of Public Health's Public Health Clinic. A listing is located here [Public Health Clinics | South Carolina Department of Public Health](#).

3. Annual TB Risk Assessment

Children's attendant caregivers must submit a [Health Care Personnel \(HCP\) Baseline Individual TB Risk Assessment annually](#) to remain eligible to serve as a children's attendant caregiver. Forty-five (45) days prior to the expiration date of the annual risk assessment, the University of South Carolina Center for Disability Resources (USC-CDR) licensed nurse (USC nurse) must notify active enrolled children's attendant caregivers of the expiration of their annual TB risk assessment. Children's attendant caregivers

are considered at increased risk for TB if any of the statements on the risk assessment are marked "Yes." Children's Attendant Caregivers determined on the assessment to be at risk for TB must provide evidence of a single BAMT or a baseline two-step Tuberculin Skin Test with the annual risk assessment. If the children's attendant caregiver does not submit the annual TB risk assessment, the USC nurse must notify SCDHHS. Current services of the children's attendant caregiver must be terminated after reasonable notice (two (2) weeks) to participants has been given so participants can find replacement services. The HCBS Provider Oversight must suspend new referrals to the children's attendant caregiver effective on the date suspension is submitted.

Please refer to the Department of Public Health (DPH) website for PPD Tuberculin test requirements.

<https://dph.sc.gov/sites/scdph/files/Library/Regulations/R.61-122.pdf>
If additional information is required, contact the Tuberculosis Control Division, Department of Public Health, 400 Otarre Parkway, Cayce, SC 29033-3751 phone (803) 898-0558.

2. Legally responsible individuals are allowed to receive payment for children's attendant care services when the MCC waiver participant is determined to require extraordinary care.
 - a. Legally responsible individuals include the following family members:
 - i. A parent of a minor Medicaid participant;
 - ii. A stepparent of a minor Medicaid participant;
 - iii. A foster parent of a minor Medicaid participant;
 - iv. A person who has the legal responsibility of utilizing their own assets for the care of the Medicaid participant (legally responsible adult)
 - b. Extraordinary care exceeds the range of tasks a family member or responsible person would normally perform in the household for a person without a disability or chronic illness of the same age. The care is necessary to ensure the health and welfare of the participant and avoid institutionalization.
 - c. Reimbursable children's attendant care services are limited to those identified by the licensed professional on the SCDHHS Children's In- Home Care Prescriber Form and must be rendered in the scope, frequency, and duration as identified and authorized

on the service plan. Only authorized services rendered by an approved children's attendant caregiver and confirmed by electronic visit verification (EVV) are eligible for reimbursement.

3. More than one enrolled children's attendant caregiver may be authorized to provide the assessed number of hours of children's attendant care to the MCC waiver participant. However, children's attendant care services from each children's attendant caregiver may not be provided at the same time.
4. When children's attendant care services are authorized for more than one MCC participant in the same home the children's attendant caregiver cannot be clocked in for both participants at the same time. The children's attendant caregiver must clock in and out separately for each participant.
5. Involuntary Termination
 - a. Participants may be involuntarily terminated from the use of children's attendant care services when they are unable to direct their own care and have no responsible party willing and/or able to do so. Participants who are involuntarily terminated from children's attendant care services are given the option of receiving agency directed services provided by an in-home care agency.
 - b. If a participant is involuntarily terminated from children's attendant care services, the termination of the service and the authorization of in-home care agency services are coordinated to assure continuity of services. These safeguards exist to assure participant health and welfare during the transition period of an involuntary termination of children's attendant care services.
 - c. Participants who are involuntarily terminated from children's attendant care services are given written appeal rights.

B. Minimum Training Requirements

Prior to the provision of services children's attendant caregivers must meet the following training requirements. The USC nurse will review documentation certifying completion of training by a SCDHHS-approved party. Copies of required training completion documents shall be maintained in the USC nurse file for each children's attendant caregiver. Documentation must remain current in order to provide children's attendant care services.

1. Cardiopulmonary Resuscitation (CPR) Training

Completion of CPR training is required. Documentation must include training date, course title, location/modality, duration of training, name of children's attendant caregiver, name of instructor/organization, and license if applicable. The documentation must be generated by an organization, or an individual delegated by an organization to provide the instruction. CPR recertification requirements may vary by the issuing authority. The American Red Cross CPR certification is valid for 2 years.

2. First Aid Training

Completion of First Aid training is required. Documentation must include training date, course title, location/modality, duration of training, name of children's attendant caregiver, name of instructor/organization, and license if applicable. The documentation must be generated by an organization, or an individual delegated by an organization to provide the instruction. First Aid recertification requirements and the frequency vary by the issuing authority and the individual needs. The American Red Cross First Aid certification is valid for 2 years.

3. Additional Skilled Care Training

For any task that has been identified as a skilled need on the SCDHHS Children's In-Home Care Prescriber Information Form, children's attendant caregivers are required to demonstrate competency in performing the identified tasks. Only the tasks identified on the SCDHHS Children's In-Home Care Prescriber Information Form will be added to the service plan. Enrolled children's attendant caregivers are only reimbursed for services outlined in the service plan.

4. Electronic Visit Verification System/Billing Training

Prior to the first authorization being issued, all children's attendant caregivers who have been matched with their first participant are required to complete Electronic Visit Verification (EVV) System/billing training.

5. Participant-specific Training

Participant-specific training for the children's attendant caregiver and/or participant/ responsible party may be provided as deemed necessary based on the professional judgment of the USC nurse or when the participant/responsible party or children's attendant caregiver requests assistance with training related to specific tasks.

Note: Training may be furnished by the USC nurse while the children's attendant caregiver is furnishing care to the participant. During the match

visit, the USC nurse may also identify additional training needs and assist the children's attendant caregiver with locating a training resource to address those needs.

C. Employer of Record

1. The provision of children's attendant care and the children's attendant caregiver must be supervised by the EOR to ensure that the employee (children's attendant caregiver) provides services as outlined in the authorization. The EOR can be the participant (aged 18 or older), responsible party, or other identified individual. The safety and efficacy of the EOR must be certified by the USC nurse. The USC nurse must determine when the EOR is no longer certified to provide supervision for the children's attendant caregiver.
2. The USC nurse and the MCC waiver nurse case manager must document when the requirement for an EOR cannot be met and is, therefore, waived. When the participant or responsible party is unable to identify a separate, qualified individual to serve as the EOR, the responsible party may be allowed to serve as the EOR and the children's attendant caregiver. Requests for exceptions must be submitted by the MCC Waiver Nurse Case Manager to WES@scdhhs.gov. All the following conditions must be met:
 - a. Participant is a minor.
 - b. Participant is a minor and the legally responsible individual is also the approved children's attendant caregiver.
 - c. The legally responsible individual / children's attendant caregiver is unable to identify a separate, qualified individual to serve as the EOR.

D. Infection Control

The children's attendant caregiver must always adhere to basic infection control procedures while providing children's attendant care services.

E. Description of Services to be Provided

1. The number of children's attendant care service hours are determined by the SCDHHS validated assessment for in-home personal care hours.
 - a. For skilled care the Nursing Acuity Scale will be used to determine the assessed number of hours needed to meet the skilled needs identified on the SCDHHS Children's In-Home Care Prescriber Information Form.
 - b. For children under five refer to the Guidelines for Developmental Stages.

- c. The assessor will compare the assessed skills on the Nursing Acuity Scale to the needs on the Children's In-Home Care Prescriber Information Form to determine which tasks can be delegated to the children's attendant caregiver.
2. The unit of service is authorized in one (1) hour increments and must consist of direct children's attendant care services provided in the participant's home. Escorting the participant to Medicaid covered services when transportation is provided by another individual is reimbursable.

Exceptions: A secondary home setting may be identified as billable place of service for children's attendant care services when approved. The MCC waiver nurse case manager must email WES@scdhhs.gov to request approval.

3. The allowable tasks provided to the child are dependent upon the individual participant's needs as set forth in the participant's approved service plan. Services must be participant specific and for the direct benefit of the participant.
4. If the assessed need for services exceeds 40 hours per week, every effort must be made to identify a separate children's attendant caregiver to provide the services for over 40 hours per week. A week is defined as Sunday – Saturday.
 - a. Children's attendant caregivers may not provide more than 16 hours of care per day.
 - b. Children's attendant caregivers may not provide more than 40 hours of care per week.
5. If multiple attendants are required, more than one attendant cannot be clocked into the EVV system at one time.
6. If a children's attendant caregiver is authorized to provide services for more than one participant, every effort must be made to ensure that the children's attendant caregiver does not exceed 40 hours per week for the combined hours for all participants.

Participants/Responsible parties may select an agency-based children's personal care provider to provide coverage for a portion of the assessed hours. In this instance, the children's personal care provider will be held to the service limitations identified in the Children's Personal Care Scope of Services and associated billing guidelines.

7. Children's attendant care services include:

- a. Extraordinary care which exceeds the range of tasks a family member or responsible person would normally perform in the household for a person without a disability or chronic illness of the same age which are required to assist the participant with activities of daily living (e.g., assistance with bathing, dressing, eating, hygiene, mobility, toileting, and transferring).
- b. Delegated skilled tasks specific to the participant's medical condition(s). Skilled tasks must be identified by the prescriber and documented on the SCDHHS Children's In-Home Care Prescriber Information Form. The USC nurse must certify competency of the children's attendant caregiver to provide identified skills via a match visit in the participant's home.
- c. Transportation is excluded from this service. The non-emergency medical transportation (NEMT) program provides transportation to and from Medicaid covered services.
- d. Escorting participant to Medicaid covered services.

Children's attendant caregivers may be reimbursed for escorting the participant to Medicaid covered services when transportation is provided by another individual. The request must be communicated to the MCC waiver nurse case manager and added to the participant's service plan. Children's attendant caregivers are prohibited from clocking in when care is not being rendered directly to the participant (e.g. when children's attendant caregiver is driving the participant to a Medicaid covered service). Children's attendant caregivers will not be reimbursed for escorting participants other than described above.

F. Record Keeping

1. The children's attendant caregiver shall maintain a copy of the following and ensure information remains confidential per SCDHHS Healthy Connections Medicaid requirements for providers:
 - a. Current and historical Service Provision Forms specifying units and services/duties to be provided;
 - b. The HCBS participant's service plan;
 - c. The children's attendant caregiver must enter the children's attendant care corresponding activity code in the EVV system. These codes must be entered at the conclusion of the service.

- d. A copy of the participant's back-up plan for service provision when the primary children's attendant caregiver is unable to provide services. (The participant/responsible party must make prior arrangements with family members, other formal or informal supports or another enrolled children's attendant caregiver for care provision in the absence of the primary children's attendant caregiver).
2. The MCC waiver nurse case manager shall monitor the children's attendant care services in accordance with roles and responsibilities outlined in the executed Care Service Organization contract. The monitoring and oversight shall include the following:
 - a. Monthly review of children's attendant care EVV entries;
 - b. Monthly documentation of oversight of the children's attendant care services rendered to the participant in the Phoenix case management system;
 - c. Completion of a quarterly face-to-face visit with the children's attendant caregiver and the participant in the home; this visit may be completed in conjunction with the mandated quarterly care coordination/case management in-person visit.

Note: MCC waiver nurse case manager must document the completion of this requirement with the initiation and completion of a children's attendant care narrative entry in the Phoenix case management system.

G. Conduct of Services

1. The children's attendant caregiver must initiate children's attendant care services on the date agreed upon by the participant/responsible party, children's attendant caregiver, EOR and the MCC waiver nurse case manager. This date must be the start date on the written authorization for services. Services provided prior to the authorized start date as stated on the Service Provision Form will not be reimbursed.
2. The MCC waiver nurse case manager must authorize children's attendant care services by designating the authorized units of services in accordance with the participant's service plan. The children's attendant caregiver must provide services per the needs outlined in the service plan. The participant and/or responsible party must self-direct the provision of care and coordinate with the children's attendant caregiver for the time for service delivery and specific tasks to be performed. If the children's attendant caregiver does not reside with the participant, the amount of time authorized does not include transportation time to and from the participant's home.

If the children's attendant caregiver or the participant/responsible party identifies children's attendant care duties that would be beneficial to the participant's care but are not specified in the participant's service plan, the children's attendant caregiver or participant/responsible party must contact the MCC waiver nurse case manager to discuss the possibility of having these duties included in the service plan. The decision to modify the duties to be performed by the children's attendant caregiver is the responsibility of the MCC waiver nurse case manager and may include collaboration with the licensed professional providing care to the child.

- a. The children's attendant caregiver must notify the MCC waiver nurse case manager immediately of the following participant changes:
 - i. Participant's condition has changed, and the service plan no longer meets participant's need, or the participant no longer appears to need children's attendant care services.
 - ii. Participant/responsible party no longer appears capable of providing supervision for the children's attendant caregiver.
 - iii. Participant/responsible party no longer wants to serve as EOR/responsible party.
 - iv. Participant dies.
 - v. Participant moves out of the service area.
 - vi. Participant/responsible party no longer wishes to receive children's attendant care services.
 - vii. Knowledge of the participant's Medicaid ineligibility or potential ineligibility
- b. The children's attendant caregiver, EOR and any other individual involved in the provision or oversight of the service must notify Child Protective Services (for MCC participants under 18 years of age) or Adult Protective Services (for MCC participants aged 18 and older) if he/she has knowledge of, or reason to believe that the participant has been or is likely to be abused, neglected or exploited within 24 hours or the next business day of becoming aware/witnessing the event. If a report is made of abuse, neglect, or exploitation, the children's attendant caregiver must notify the MCC waiver nurse case manager within 24 hours or the next business day of when the report is made.
- c. The participant must have an effective back-up service provision plan in place to ensure that the participant receives services in the absence of the primary children's attendant caregiver.

- d. When the children's attendant caregiver determines that services cannot be provided by the children's attendant caregiver as authorized, the children's attendant caregiver must immediately notify the MCC waiver nurse case manager and the participant/responsible party by telephone.
- e. When two consecutive attempted visits occur, the children's attendant caregiver/participant/responsible party or EOR must notify the MCC waiver nurse immediately. An attempted visit occurs when the children's attendant caregiver arrives at the home and is unable to provide the assigned tasks because the participant is not at home, or the service has been refused.
- f. The children's attendant caregiver must notify the MCC waiver nurse case manager immediately if the children's attendant caregiver wishes to terminate as the provider.

When applicable, during the match visit the USC nurse is responsible for giving participants a written description of the state law concerning advance directives in accordance with the Patient Self-Determination Act. The USC nurse must assist children's attendant caregivers in meeting this requirement.

- g. The children's attendant caregiver shall adhere to all SCDHHS policies, procedures and Medicaid provider manuals including policies regarding billing, claims adjustments, financial management services vendor requirements, etc.
- h. The children's attendant caregiver must comply with all Electronic Visit Verification (EVV) System requirements for all participants.

H. Skilled Services Provided by an Approved Children's Attendant Caregiver

The waiver service definition for children's attendant care allows a participant or the designee to self-direct "skilled service activities" performed by the children's attendant caregiver with approval from the participant's prescriber. These activities are basic and ancillary services that enable an individual in need of in-home care to live in the individual's home and community rather than in an institution and to carry out functions of daily living. With prescriber's approval, children's attendant caregivers may provide skilled services which are considered basic and ancillary care for participants in the MCC waiver.

When a referral is made to the USC nurse for children's attendant care that includes a request to provide skilled service activities, the MCC waiver nurse case manager must include a comment on the referral stating "Participant/responsible party has requested a skilled service to be provided."

For existing referrals already in process with the USC nurse where a skilled service need has been identified, the MCC waiver nurse case manager must note in the comment section on the referral, *"Participant/responsible party has requested a skilled service to be provided"* AND email the USC nurse to notify them of the change in the referral. The USC nurse will not receive a Phoenix notification that there is a change to the referral.

The USC nurse will notify the assigned MCC waiver nurse case manager when the "SCDHHS Children's In-Home Care Prescriber Information Form" can be sent to the prescriber completion including signature.

Once the signed Children's In-Home Prescriber Information Form has been received, the MCC waiver nurse case manager must scan the form into Phoenix scans under the children's attendant caregiver scan tag and email the USC nurse to notify the form has been returned and scanned. The USC nurse must complete the children's attendant caregiver match visit once this notification is received by email.

The following list of services are examples of activities which may be appropriate for the children's attendant caregiver to perform. *This list is not all inclusive.*

- Tracheostomy care
- Ventilator Care - taking on and off the vent
- Suctioning- tracheostomy/ oral
- Finger stick blood glucose monitoring
- Wound care
- Enteral feedings (PEG or Nasogastric)
- Bowel program
- Ostomy care
- Catheter care

Only skills on the Children's In-Home Care Prescriber Information Form can be rendered by the approved children's attendant caregiver and reimbursed by SCDHHS.

NOTE: If the participant changes physician, nurse practitioner, or physician assistant and the prescriber who signed the Children's In-Home Prescriber Information Form is no longer involved in the participant's care, a new Children's In-Home Prescriber Information Form is required.

If the service is directed by the EOR, and the EOR is no longer able to direct the skilled service activities performed by the children's attendant caregiver, a new Children's In-Home Care Prescriber Information Form statement is required.